

Unit 3

Activity and Exercise Pattern

- ❑ The wide range of physical activities that people initiate and perform to maintain life, health and well-being
- ❑ Three areas may be assessed in this pattern:

Musculoskeletal function

Respiratory function

Cardiovascular functions

Nursing activities involved in this pattern

2

1- Care of unit environment (d/t bed making)

- Bed making: is usual procedure to change bed linen after bath or soiling
- **Purpose:** To make smooth and comfortable bed with list disturbance of the patient

Fundamentals of bed making

- When removing sheet from the bed lift the mattress, while loosening the bed linen never pool sheet with force
- Turn the mattress from top to bottom or from side to side when making un occupied bed

2- Providing Perineal Care

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- Is cleansing of the external genitalia, perineum, and surrounding area
- **Purposes:-** to prevent or eliminate infection and odor,
 - to promote healing,
 - to remove secretions, and provide comfort
- Perineal care can be provided alone or as part of the bed bath
- It may be an embarrassing procedure for both the client & the nurse, especially if the client is of the opposite sex
- Clients who are embarrassed may elect to perform their own perineal care

3- Providing Oral Care

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- ❑ The oral cavity functions in mastication, secretion of mucus to moisten and lubricate the digestive system, and secretion of digestive enzymes.
- ❑ Common problems occurring in the oral cavity are:
 - Bad breath (halitosis)
 - Dental caries (cavities)
 - Plaque
 - Periodontal disease (pyorrhea)
 - Inflammation of the gums (gingivitis)
 - Inflammation of the oral mucosa (stomatitis)

4- Offering bed pan and urinals

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Equipment –

- ☐ Bed pan or urinal and cover
- ☐ Toilet paper
- ☐ Basin with soap and water may or may not be necessary
- ☐ Charting material
- ☐ Glove

5- Providing Bathing Needs

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- ❑ Bathing of clients is an essential component of nursing care
- ❑ The nurse retains the responsibility for assuring the hygienic needs of the client are met
- ❑ The type of bath provided will depend on the purpose of the bath and the client's self-care ability
- ❑ The two general categories of baths are cleansing and therapeutic

6- Providing Eye, Ear, and Nose Care

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- ❑ **Eyes:** *Eyelids should be washed daily with a warm washcloth from the inner to outer canthus*
- ❑ Eyelashes and eyebrows should be washed as necessary
- ❑ Client's artificial eye may require daily cleaning, artificial eye be removed from the eye socket and washed
- ❑ Some artificial eyes are permanently implanted.

7- Applying Principles of Body Mechanics

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- ❑ To have physical strength in order to assist clients in achieving mobility
- ❑ Carrying, pulling, pushing, or lifting clients and/or equipment are all activities involved in the delivery of nursing care
- ❑ Nurses' implementation of correct body mechanics help minimize the following:
 - Clients injury
 - Nurses work-related musculoskeletal injury
 - Nurses fatigue

- ❑ Back injury is mainly caused by lifting unreasonable loads .
- ❑ The following variables can increase the risk of nurse injury:
 - Client weight
 - Client weight-bearing ability
 - Client combativeness and unpredictability
 - Height of bed
 - Confined work space
 - Wheelchairs without adjustable arms

- ❑ Educating staff about the use of proper body mechanics is essential in preventing injury
- ❑ The following recommendations are made to prevent MSDs:
 - Adjust the height of working surfaces to reduce long reaches and awkward postures
 - Reduce the weight and size of items that workers must lift
 - Provide mechanical lifting equipment

8- Keeping Alignment: Positioning

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- ❑ Clients cannot always move independently and reposition themselves in bed
- ❑ Nurse must use proper turning and positioning techniques in order to achieve the following outcomes:
 - Increase client comfort
 - Prevent contractures
 - Prevent decubitus (pressure sores)
 - Make portions of the client's body accessible for procedures
 - Help clients access their environment

- ❑ Clients who cannot move independently must be repositioned every 2 hours
- ❑ Repositioning must be done more often for clients who are:-
 - Uncomfortable or incontinent, or
 - Who have fragile skin,
 - Poor circulation, fragile skin,
 - Decreased sensation,
 - Poor nutritional status, or impaired mental status

- Nurses need to be aware of three essential concepts when positioning clients: **pressure, friction, and skin shear**
- **A pressure** site is any skin surface area on which the client is lying or sitting
- Its force can compromise circulation and lead to skin breakdown and ulceration
- Prominences are more likely to experience impaired skin integrity, so observe the area/body for any prominence

- ❑ **Friction:** is caused when the skin is dragged across a rough surface such as bed sheets or stretcher surfaces
- ❑ it causes heat, which damages the skin and may lead to decreased skin integrity with resultant infection and/or skin breakdown.
- ❑ **Skin shear:** is the result of dragging skin across a hard surface.
- ❑ The force of resistance to being dragged tears the deep layers of skin which can lead to skin ulceration.
- ❑ Practicing Proper Body Mechanics- **see procedure**

❑ ***Positioning a Client in Bed***

- Fowler's Position, Supine Position
- Side-Lying Position
- Logrolling a Client

❑ ***Transferring a Client from Bed to Chair***

- Bed to stretcher
- Bed to wheel chair

-see these steps from fundamental of nursing-I

9- Performing Range-of-Motion Exercises

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- ❑ Ability of joints to extent they can move
- ❑ It may be active or passive ROM
- ❑ Passive ROM- assisted by nurse
- ❑ Active ROM- self exercised applied by patient
- ❑ **Purpose:**
 - ❑ To maintain full flexibility,
 - ❑ To maintain muscle tone and strength,
 - ❑ To prevent contractures, and
 - ❑ To improve circulation

Application of Nursing Process

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- ***This pattern involves assessment of three areas:***
- Mobility and activity: self care & hygiene, body mechanics & mobility
- Respiratory function, and
- Cardiac function

Assessment

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- Activity and exercise is important for normal functioning of the RS, CVS and MSK systems

i- Mobility and activity:

- **Subjective data on mobility and self care:**
 - Activity of daily living (self care skills, bathing toileting, dressing, feeding, etc)
 - Simple motor activities (sitting, standing, walking)
 - Home maintenances skills (cooking, shopping, bed making, washing dishes)
 - Any restriction of activity or exercise

- ❑ **Objective data (Musculoskeletal ass't)**
- ❑ Musculoskeletal assessment
- ❑ Motor examination (gait and balance, coordination, abnormal movements, stance muscle characteristics (tone, strength, bulk) and body position)
- ❑ Joint mobility, muscle strength

ii- Respiratory function:

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- ❑ **Subjective data on respiration**
 - Sign and symptom of respiratory disease such as cough, chest pain
 - Risk factors for lung disease such as smoking, exposure to pollutants
- ❑ **Objective data – respiratory pattern ass't**
- ❑ Respiratory pattern and assessment of lung and thorax

iii- Cardiovascular function

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- ❑ **Subjective data on cardiovascular function**
- ❑ Risk factors for cardiovascular disease such as family history, smoking, elevated cholesterol, hypertension, etc.
- ❑ Signs of cardiovascular dysfunction such as fainting, palpitation, dizziness, etc
- ❑ Impact of the cardiovascular problem on functioning
- ❑ Specific adaptations of patient to the problem
- ❑ Medications.
- ❑ **Objective data – cardiovascular pattern ass't**
- ❑ Assessment of cardiac and vascular system

Nursing Diagnosis

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- Self care deficit
- *Defining characteristics*
 - ▣ Unable to move
 - ▣ Unable to feed

Planning

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- **Specific client goals are:**

Nursing Intervention

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- *Nursing intervention to promote self care activity*

Evaluation

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- The nurse evaluates the degree to which self care impairment has been resolved according to the client goals initially established.

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3.1:

GENERAL CARE OF THE PATIENT

Learning Objectives

At the end of this unit, the learner able to:

- Describe several aspects of general care to the patient, including bath, oral, hair, perineal area care, giving and receiving bedpan urinals; and feeding helpless patient.
- Explain the rationale for each patient care procedure.
- Identify and list the materials required to do these procedures.
- Describe the procedure following a logical order
- Demonstrate the ability to perform each of these specific care

Bath (Bathing and Skin Care)

- Bathing of clients is an essential component of nursing care
- Whether the nurse performs the bath or delegates the activity to another health care provider, the nurse retains the responsibility for assuring that the hygienic needs of the client are met.
- The type of bath provided will depend on the purpose of the bath and the client's self-care ability.
- The two general categories of baths are cleaning and therapeutic

Cleansing Baths

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- **Cleansing bath:** Is given chiefly for cleansing or hygiene purposes and includes:
 - ✓ **Complete bed bath:** the nurse washes the entire body of a dependent patient in bed
 - ✓ **Partial bath (abbreviated bath):** only the parts of the client's body that might cause discomfort or odor, if neglected are washed the face, hands, axilla, perineum and back (the nurse can assist by washing the back) omitted are the arms, chest, abdomen.

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Self-help bed bath: clients confined to bed are able to bath themselves with help from the nurse for washing the back and perhaps the face

- ✓ **Tub bath:** preferred to bed baths because it is easier to wash and rinse in a tub. Also used for therapeutic baths
- ✓ **Shower:** many ambulatory clients are able to use shower

Therapeutic Bath

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- Therapeutic baths require a physician's order stating the type of bath, temperature of water, body surface to be treated, and the type of medicated solutions to use.
- A therapeutic bath is usually performed in a tub and lasts about 20 to 30 minutes.
- Therapeutic baths are classified as hot or warm water, cool or tepid water, soak, sitz, depending on the prescribed type of bath.
- **Hot- or warm-water tub baths** are used to reduce muscle spasms, soreness, and tension.
- **Cool or tepid baths** are used to relieve tension or lower body temperature.

Sitz bath

- It is used to sock the client's pelvic area.
- It cleanse and reduce inflammation in the perineal and anal areas
- Only the client's pelvic area is immersed in warm fluid; the client sits in a special tub or chair or in a basin placed on the toilet seat so that the legs and feet remain out of the water.
- Used for clients who have had rectal surgeries, an episiotomy during childbirth, painful hemorrhoids, or vaginal inflammation

Purpose:

- ❑ To relieve pain in post operative rectal condition
- ❑ Smoothen irritated skin (perineum)
- ❑ Facilitates wound healing (after episiotomy)
- ❑ To release the bladder in case of urinary retention

Procedure

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- A client sits in a special tub or a bowel
- The area from the mid thighs to the iliac crests or umbilicus - increases circulation to the perineum (when the legs are also immersed blood circulation to the perineum or pelvic area decrease)
- If it is going to be given in the tub – fill $\frac{1}{2}$ the tub with water and add the ordered medication
- In a bowel – fill $\frac{2}{3}$ of it with water – add the ordered medication and dilute

- Water temperature should be from 40° to 43°C (105° to 110°F).
- Prevent overexposure and chilling by draping a bath blanket over the client's shoulders and thighs, and prevent drafts.
- The medication to Rx the perineum in KMNO₄ soln 250 mg KMNO₄ in 500 ml of water

- The duration of the bath is generally 15-20 minutes (20-25) depending on the client's health
- Assess the client during the bath for extensive vasodilatation, faintness, dizziness, weakness, increased pulse rate, and pallor
- Help the client to dry.

Before bathing a patient, determine

- The type of bath the client needs
- What assistance the client needs
- Other care the client is receiving – to prevent undue fatigue
- The bed linen required

Principles of Bath

- The water should feel comfortably warm for the client
- People vary in their sensitivity to heat generally it should be 43-46°C (110-115°F)
- The water for a bed bath should be changed at least once
- Close doors and windows: air current increases loss of heat from the body by convection

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- Provide privacy – hygiene is a personal matter & the patient will be more comfortable
- The client will be more comfortable after voiding and voiding before cleansing the perineum is advisable
- Place the bed in the high position: avoids undue strain on the nurses back
- Assist the client to move near you -facilitates access which avoids undue reaching and straining

- Make a bath mitt with the washcloth. It retains water & heat better than a cloth loosely held
- Clean the eye from the inner canthus to the outer using separate corners of the wash cloth – prevents transmitting micro organisms, prevents secretions from entering the nasolacrimal duct
- Firm strokes from distal to proximal parts of the extremities increases venous blood return
- **Note: *when bathing a client with infection, the caregiver should wear gloves in the presence of body fluids or open lesion***

Giving bed bath

Definition

- Bed bath is a bath given to a patient in bed who is unable to care for himself. Also called sponge bath, blanket bath

Purpose:

- To prevent bacteria spreading on skin
- To promote relaxation, comfort and cleanliness and to induce sleep
- To stimulate the circulation and improve general muscular tone& joint
- To observe skin condition and objective symptoms
- To prevent body odors and pressure sores.

Equipments Required

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- Trolley (1), screen (1)
- Bath towel (2) and Mackintosh (1): (A) to covering over mackintosh (1) (B) for covering over client's body and drying (1)
- Basin (2): for without soap (1) for with soap (1) and Jug (1)
- Bucket (2): for clean hot water (1) for waste (1)
- Wash (Sponge) cloth (2): for wash with soap (1) for rinse (1)
- Additional bed linens for making bed(one or two large sheets as needed ,pillow case and clean pajamas or gown)
- Bath blanket if available or use top sheet as bath blanket

- ☐ Disposable gloves
- ☐ Soap with soap dish (1)
- ☐ Galipot with Gauze or cotton piece
- ☐ Lotion thermometer (if available) (1)
- ☐ bed pan or urinal if needed
- ☐ tray containing items for back care, nail care , mouth care and hair care like brush and comb
- ☐ Hygienic supplies, such as, lotion, powder or deodorants (if required)
- ☐ Hamper for soiled cloths outside the room

Procedure

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- Explain the purpose and procedure to the client. Confirm any orders. Check client identification and condition. If he or she is alert or oriented, question the client about personal hygiene preferences and ability to assist with the bath.
- Wash your hands and clean the trolley from top to bottom and from far to near. Gather all required equipments. Bring all equipments to bed-side. Push trolley inform you to keep the equipment from falling and contamination. Close the curtain or the door. Put the screen to provide privacy.

- ❑ Before starting bath offer bed pan or urinal and removal bed pan or urinal. Empty it.
- ❑ Wash your hands and put on gloves
- ❑ Remove top bedding fold and place over back of chair. If there is no chair, it should be placed on foot of bed.

- Assist the client to move toward the side of the bed where you will be working. Usually you will do most work with your dominant hand.
- Remove gown, remove pillow unless this is uncomfortable for patient pillow for turning patient if necessary otherwise remove soiled pillow case place soiled gown in it and place it on chair or foot of bed between mattress and foot of bed.

Cont'd...

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- Cover the client's body with a bath blanket if available or use top sheet.
- If an IV is present on the client's upper extremity, thread the IV tubing and bag through the sleeve of the soiled cloth.
- Re-hang the IV solution. Check the IV flow rate.



- Fill two basins about two-thirds full with warm water (43-46°C or 110-115°F).
- Expose only the parts of the patient's body being washed avoid unnecessary exposing.
- Wash, rinse and dry each body parts thoroughly using washing towels and paying particular attention to skin folds.

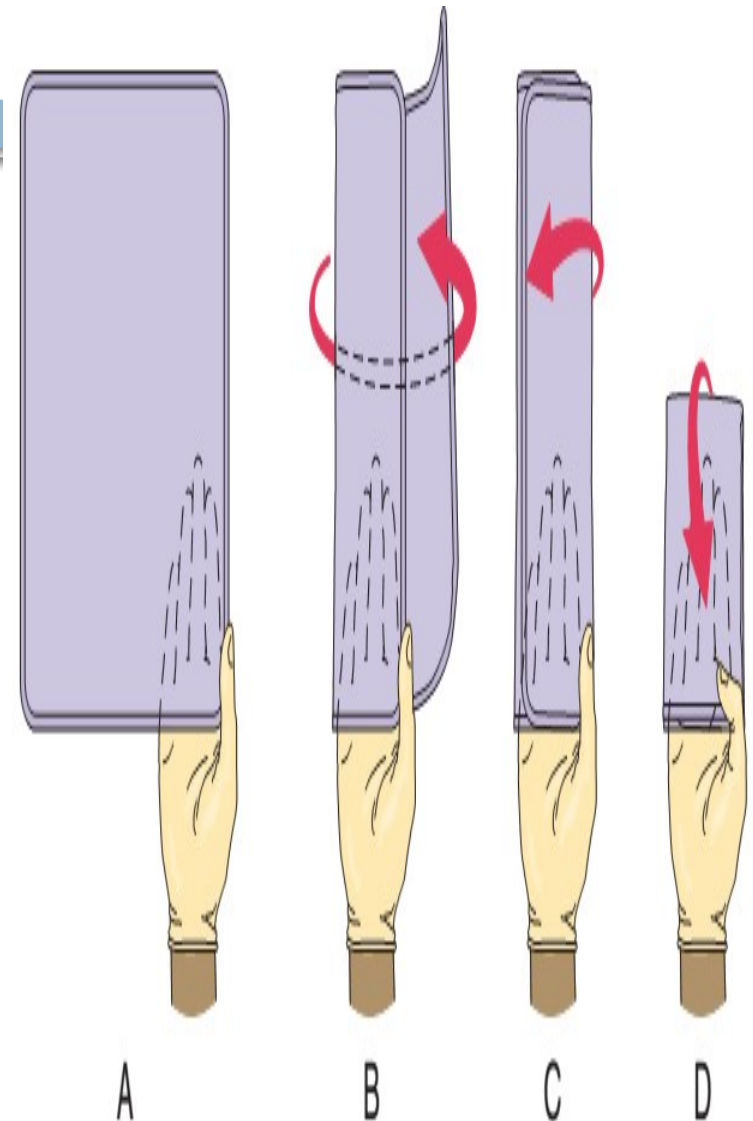
Suggested order for washing body parts:

- ☐ **Eye, Face,**
- ☐ **Ear, neck,**
- ☐ **Arms and hands farther away and then nearest to the nurse,**
- ☐ **Chest, abdomen,**
- ☐ **Leg and foot farther away and then nearest to the nurse,**
- ☐ **Back, buttocks and genital area**
- ☐ **N.B. Change the water after it gets dirty**

Cont'd....

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- **Face, neck, ears**
- Put mackintosh and big towel
Ⓐ under the client's body
from the head to shoulders.
Place face towel under the
chin which is also covered
the top sheet.
- Make a mitt with the sponge
towel and moisten with plain
water.



Cont'd....

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- Wash the client's eyes. Cleanse from inner to outer corner. Use a different section of the mitt to wash each eye.
- Wash the client's face, neck, and ears. Use soap on these areas only if the client prefers. Rinse and dry carefully.



Cont'd...

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Upper extremities

- Move the mackintosh and big towel ○ A to under the client's far arm.
- Uncover the far arm.
- Fold the sponge cloth and moisten.
- Wash the far arm with soap and rinse. Use long strokes: wrist to elbow → elbow to shoulder → axilla → hand



Cont'd...

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Place the basin on the towel.

Put the person's hand into the water. Wash it well. Clean under the fingernails with an orangewood stick or nail file.

- Dry by face towel
- Move the mackintosh and big towel ○ A to under the near arm and uncover it
- Wash, rise, and dry the near arm as same as procedure d).



Cont'd..

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- **Chest and abdomen**
- Move the mackintosh and bath towel ○A to under the upper trunk
- Put another bath towel ○B to over the chest
- Fold the sponge towel and moisten
- Wash breasts with soap and rinse. Dry by the big towel covering.



- Move the bath towel OB covering the chest to abdomen.
- Fold the sponge cloth and moisten.
- Wash abdomen with soap, rinse and dry
- Cover the trunk with top sheet and remove the bath towel OB from the abdomen

- ❑ **Exchange the warm water.**
- ❑ **Lower extremities**
- ❑ Move the mackintosh and bath towel ○A to under the far leg. Cover the near leg with bath towel ○B
- ❑ Fold the sponge cloth and moisten.
- ❑ Wash with soap, rinse and dry. Direction to wash: from foot joint to knee → from knee to hip joint.

Cont'd....

- 57 ☐ Place the basin on the towel near the foot. Lift the leg slightly. Slide the basin under the foot. Place the foot in the basin. Use an orangewood stick or nail file to clean under toenails if necessary.
- ☐ Repeat the same procedure as 15.a- c) on the near side.
- ☐ Cover the lower extremities with top sheet Remove, mackintosh and big towel.

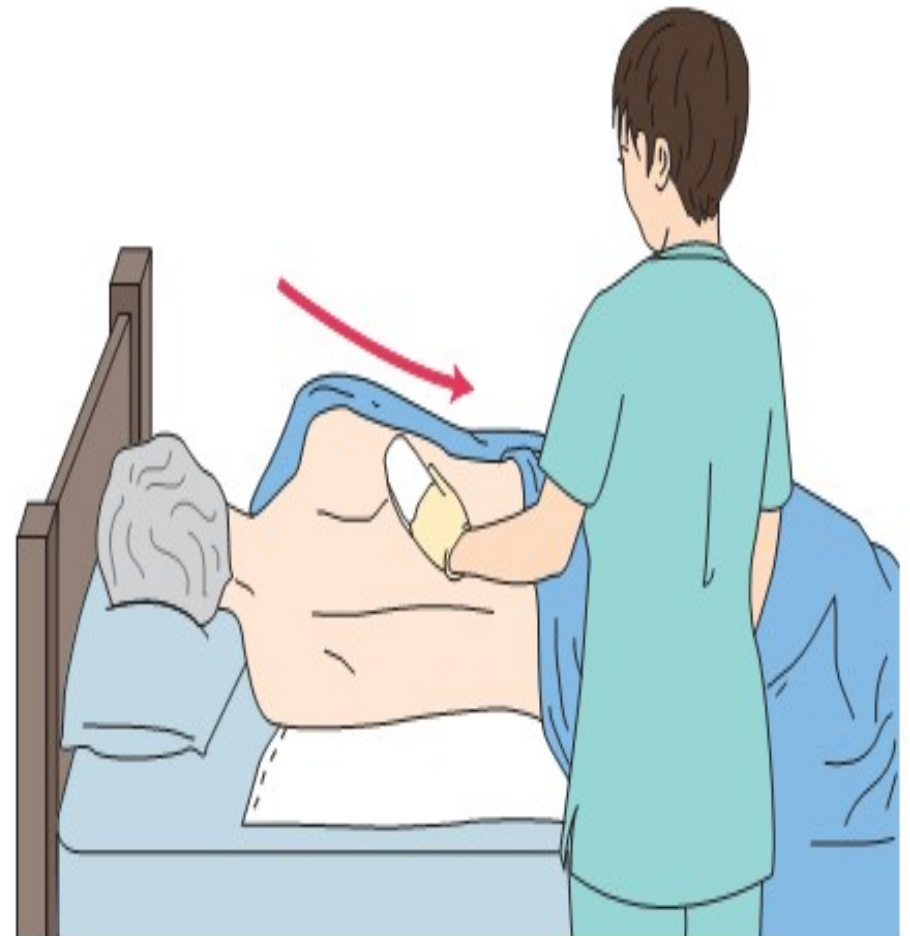


- Repeat the same procedure as 15.a- c) on the near side.
- Cover the lower extremities with top sheet
Remove, mackintosh and big towel. ○A



- **Turn the client on left lateral position with back towards you.**
- **Back and buttocks**
- Wash back. Assist client into prone or side-lying position facing away from you.
- Move the mackintosh and big towel ○A under the trunk. Cover the back with big towel ○B
- Fold the towel and moisten. Uncover the back.

- Wash the back and buttocks using long, firm strokes. Rinse and pat dry with big towel ○B.
- Give back rub and apply lotion if needed * See our nursing manual “Back Care”
- Remove the mackintosh and big towel



- Return the client to the supine position.
- **Perineal care:** Assist client to supine position. Perform perineal care. *See our nursing manual “Perineal care”
- Apply lotion and powder as desired. Assist the client to wear clean cloth. Put on clean gown protect the pillow or bed with face towel and comb patient's hair. Cut and clean finger nails and toe nails give mouth wash with plain water

- **After bed bath**
- Make the bed tidy and keep the client in comfortable position.
- Check the IV flow and maintain it with the speed prescribed if the client is given IV.
- Give care of used equipment, remove glove and wash hands,
- **Document on the chart with your signature and report any findings to senior staff.**

Giving tub bath

- ☐ Clients frequently prefer and enjoy tub baths
- ☐ A tub bath permits washing and rinsing in the tub. Tub baths can also be therapeutic.

Equipment

- ☐ Soap
- ☐ Washcloth
- ☐ Bath towel
- ☐ Gown
- ☐ Slipper
- ☐ Stool

Procedure

- Assist patient to undress
- Assist patient into the tub and avoid falling
- Allow patient to bath himself or assist as necessary
- Assist patient out of tub and dry his body and put on gown
- Return patient to room and put to bed
- Clean bath tub and leave room in order
- Discard soiled linen



Safety measures

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- ☐ Have correct temperature of water
- ☐ Avoid chilling
- ☐ Always keep bathroom unlocked

Giving back care

Definition

- Back care means cleaning and massaging back, paying special attention to pressure points.
- Especially back massage provides comfort and relaxes the client; thereby it facilitates the physical stimulation to the skin and the emotional relaxation.
- Back massage is the application of pressure and motion by the hands. It involves kneading, rubbing, and using friction

Purpose

- ❑ To improve circulation to the back
- ❑ To refresh the mode and feeling
- ❑ To relieve from fatigue, pain and stress
- ❑ To induce sleep
- ❑ To relax patient
- ❑ To prevent pressure sores or decubitus ulcer

Equipments required

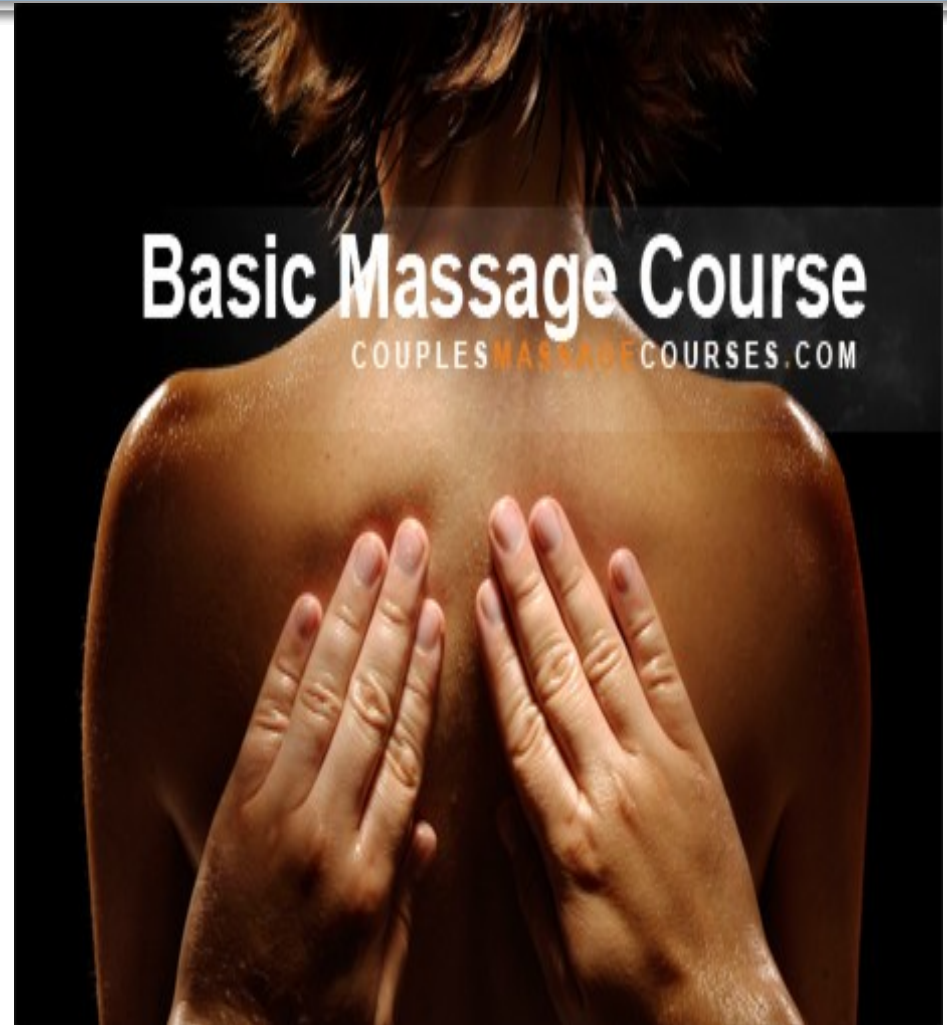
- ❑ Basin with warm water (2)
- ❑ Bucket for waste water (1)
- ❑ Sponge cloth (2): 1 for with soap 1 for rinse
- ❑ Big Towel (2): 1 for covering a mackintosh 1 for covering the body
- ❑ Oil/ Lotion/ Powder (1): according to skin condition and favor

- Gauze pieces (2)
- Soap with soap dish (1)
- Face towel (1)
- Mackintosh (1)
- Tray (1)
- Trolley (1)
- Screen (1)

Basic massage techniques

Effleurage:

- The whole hand, gliding and long rhythmic strokes are used.
- Firm, even-pressured strokes are directed toward the heart to assist blood return.
- Lighter pressure is used when moving away from the heart.



Cont'd....

71 Petrissage

- Pressing, squeezing, kneading, and rolling movements by both hands (use entire hand) are used. Deep circulation is enhanced.
- C-shaped motions stimulate the muscle body. Promotes muscle relaxation.



Cont'd...

Friction Rub

- Thumb pads, heel of hand, or fingertips are used.
- Focused, deep, circular motions are used. Penetrates deeper muscle layers.
- Is done after effleurage and petrissage



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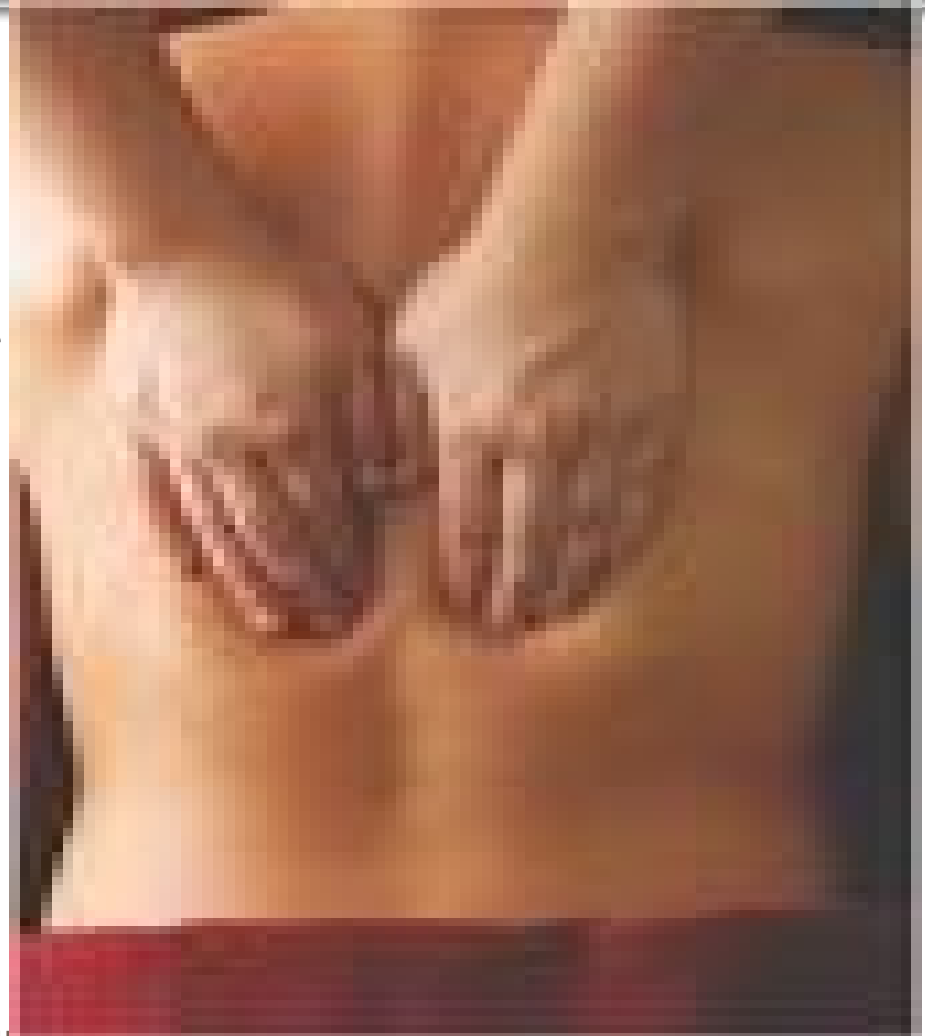
73 **Tapotement**

- Palms, fingertips, and knuckles are used.
- Brisk, vigorous, rhythmic, percussive movements are used.
- Hands alternately tap, cup, slap, and pummel muscles.
- Invigorates and stimulates tired muscles.



Vibration:

- Very fine, rapid, shaking movements are administered by the entire hand.
- Stimulates or relaxes muscles.



Procedure

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- Explain to the client about the purpose and the procedure.
- Perform hand hygiene
- Assemble all equipments required.
- Check the client's identification and condition.
- Put all required equipments to the bed-side and set up.
- Close all windows and doors, and put the screen or / and utilize the curtain if there is.

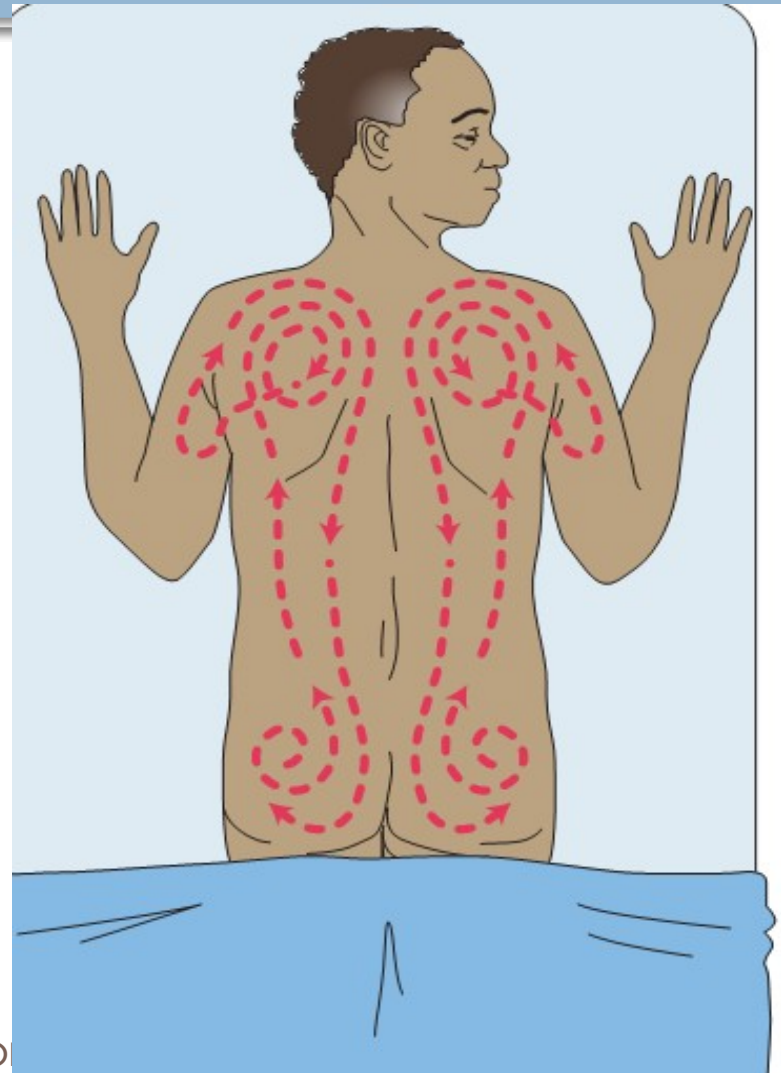
- Placing the appropriate position:
 - a. Move the client near towards you.
 - b. Turn the client to her/ his side and put the mackintosh covered by big towel under the client's body
- Expose the client's back fully and observe it whether if there are any abnormalities.

- Lather soap by sponge towel. Wipe with soap and rinse with plain warm water.
- Put some lotion or oil into your palm. Apply the oil or the lotion and massage at least 3-5 minutes by placing the palms:
 - From sacral region to neck
 - From upper shoulder to the lowest parts of buttocks
 - Begin with light to medium effleurage at lower back and continue upward following muscle groups, being careful to avoid the spine and spinal processes.

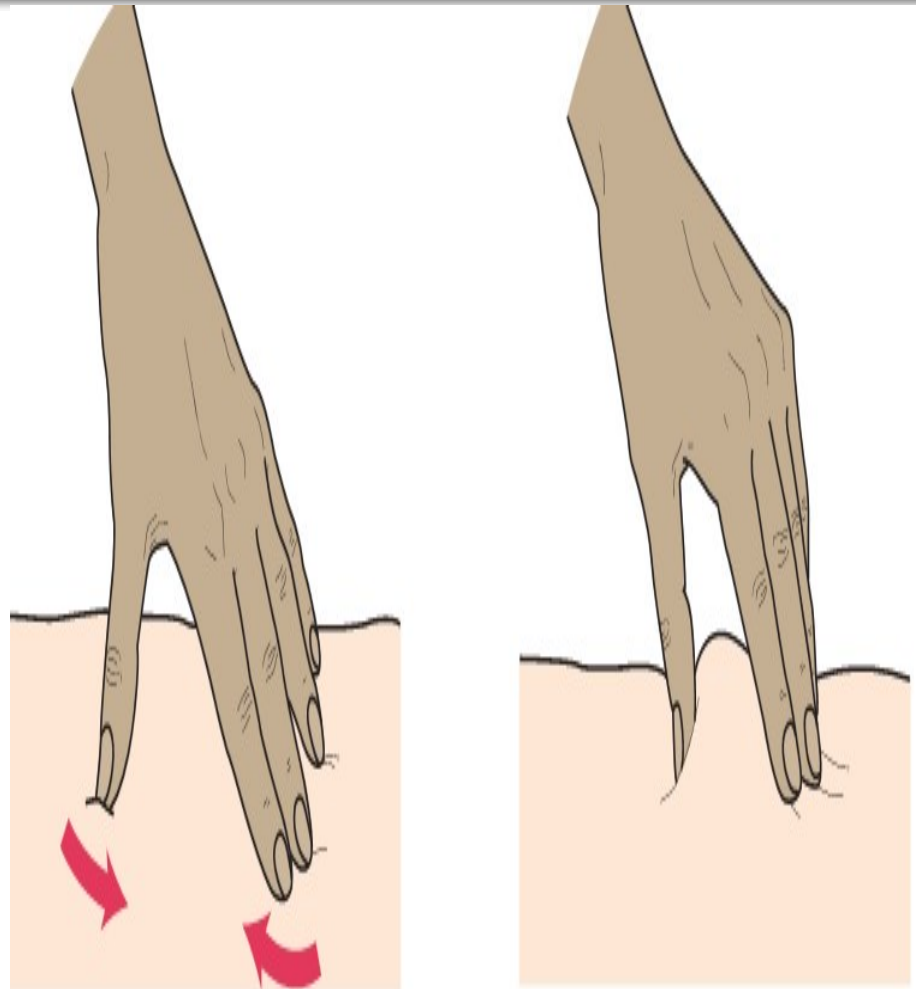
Move hands up toward the base of the neck and continue outward over the trapezius muscles with circular motions, over and around shoulders and upper arms, and return with lighter downward strokes laterally over the latissimus dorsi to the upper gluteals

- Continue the effleurage for approximately 3 minutes

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- Continue treatment if appropriate with gentle petrissage to major muscle groups in the back, shoulders, and upper arms. Use friction to particular muscle groups where tension is being held.

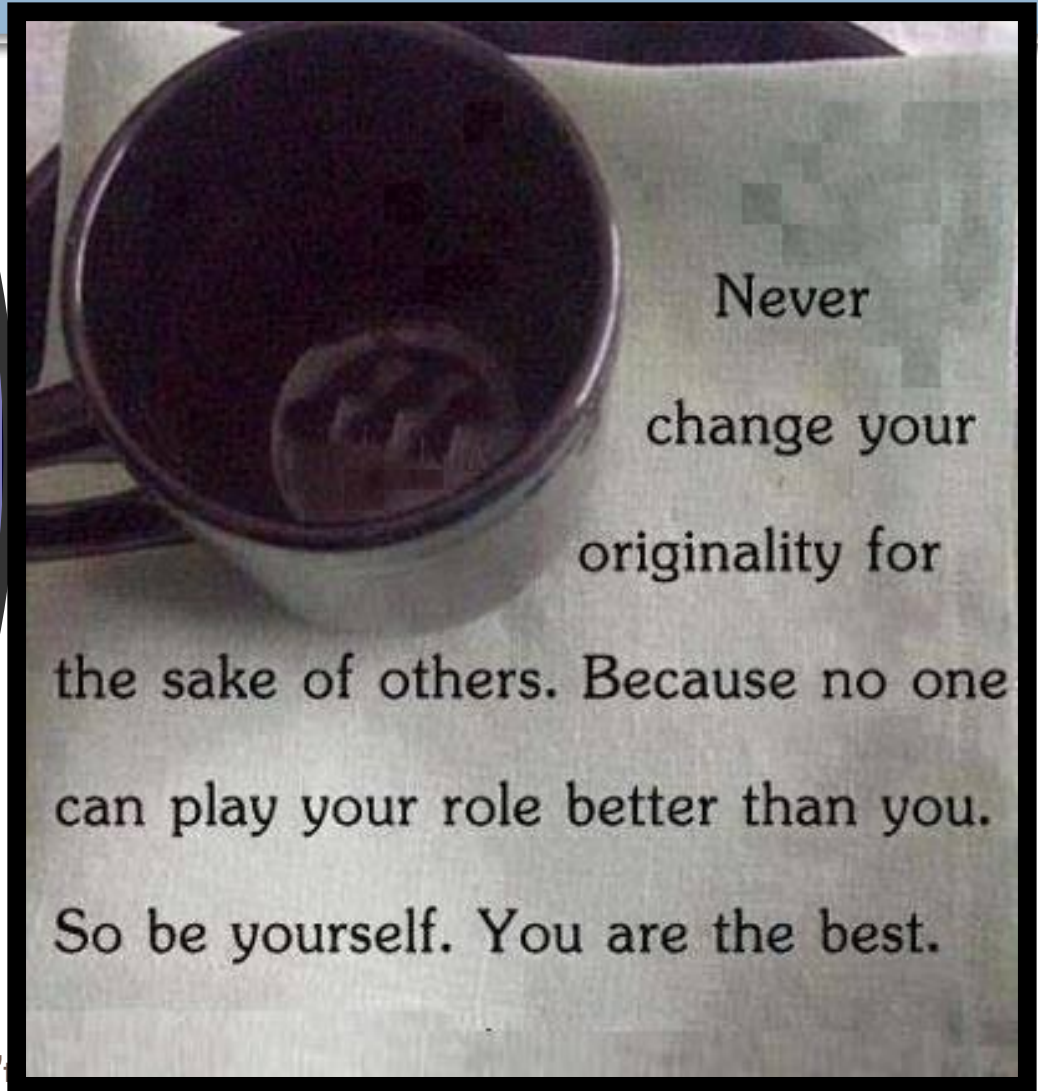


- Use tapotement to stimulate any muscle groups that may be fatigued.
- Finish treatment with effleurage.
- Wipe any excess lotion or oil from skin with towel, or use small amount of warm soap and water to clean client's skin, taking care to dry completely. Dry back and apply powder.
- Assist client into comfortable position for a period of rest or sleep.

- Help for the client to put on the clothes and return the client to comfortable position.
- Replace all equipments in proper place
- Perform hand hygiene.
- Document on the chart with your signature, including date, time and the skin condition
- Report any findings to senior staff

Have a Tea Break.....

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Giving mouth care

Definition

- Mouth care is defined as the scientific care of the teeth and mouth.
- **Purpose**
 - ▣ To keep the mucosa clean, soft, moist and intact
 - ▣ To keep the lips clean, soft, moist and intact
 - ▣ To prevent oral infections

- ▣ To remove food debris as well as dental plaque without damaging the gum
- ▣ To alleviate pain, discomfort and enhance oral intake with appetite
- ▣ To prevent halitosis or relieve it and freshen the mouth
- ▣ Daily oral care is essential to maintain the integrity of the mucous membranes, teeth, gums, and lips

Common problems occurring in the oral cavity are:

- ❑ Bad breath (halitosis)
- ❑ Dental caries (cavities)
- ❑ Plaque
- ❑ Inflammation of the gums (gingivitis)
- ❑ Inflammation of the oral mucosa (stomatitis)

Equipment

- ❑ Brushing and Flossing
- ❑ Glass of fresh water (Cup of water)
- ❑ Mouth wash solution(1.3 hydrogen peroxide solution
1 /2 tsp, salt or sodium bicarbonate to a glass or
water(250cc)
- ❑ Applicators -cotton tipped
- ❑ Tongue depressor wrapped with gauze bandage
- ❑ Emesis basin or kidney dish

- Towel and paper bag or folded paper for waste
- Lubricate liquid paraffin or mineral oil cold cream, glycerine & lemon juice, Vaseline)
- Tooth brush and tooth paste if patient has his own
Ethiopian tooth stick may be used
- Denture Care, Denture brush, Denture cleaner, Denture cup if needed
- Non-sterile gloves , Mirror , Lip moisturizer

Mouth Wash Solutions

- Normal solution: a solution of common salt with H_2O in proportion of 4 gm/500 cc of water
- Hydrogen peroxide – 5-20 cc (in water)
- $KMNO_4$ – in crystal form 4cc or $KMNO_4$ solution in a glass of water (1:700) or one small crystal in a glass of water
- Lemon juice: 2TSF lemon juice in a cup of water - an improvised method for mouth wash

Procedure

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- ❑ **Prepare the pt**
- ❑ Explain the procedure
- ❑ Assist the patient to a sitting position in bed (if the health condition permits).
- ❑ If not assist the patient to side lying with the head on pillows.
- ❑ Place the towel under the pt's chin.
- ❑ If pt confined in bed, place the basin under the pt's chin



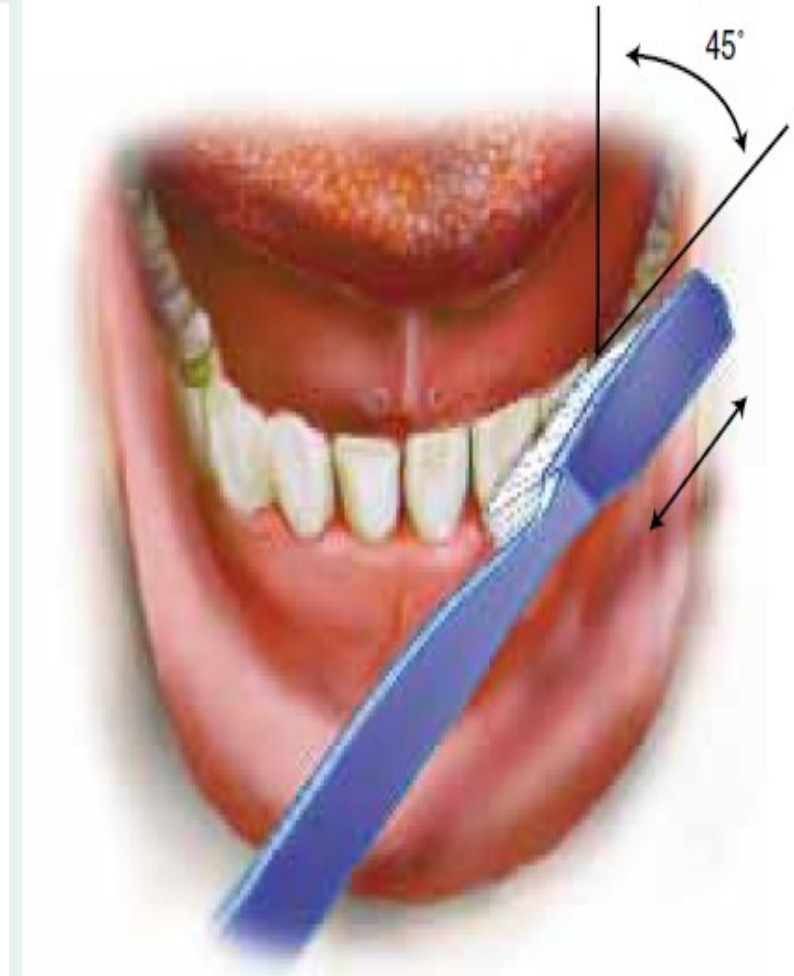
Brush the teeth

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- Moisten the tooth with water and spread small amount of tooth paste on it
- Brush the teeth following the appropriate technique.

Brushing technique

- Hold the brush against the teeth with the bristles at up degree angle.
- Use a small vibrating circular motion with the bristle at the junction of the teeth and gums use the some action on the front and the back of the teeth.
- Use back and forth motion over the biting surface of the teeth.
- Brush the tongue last



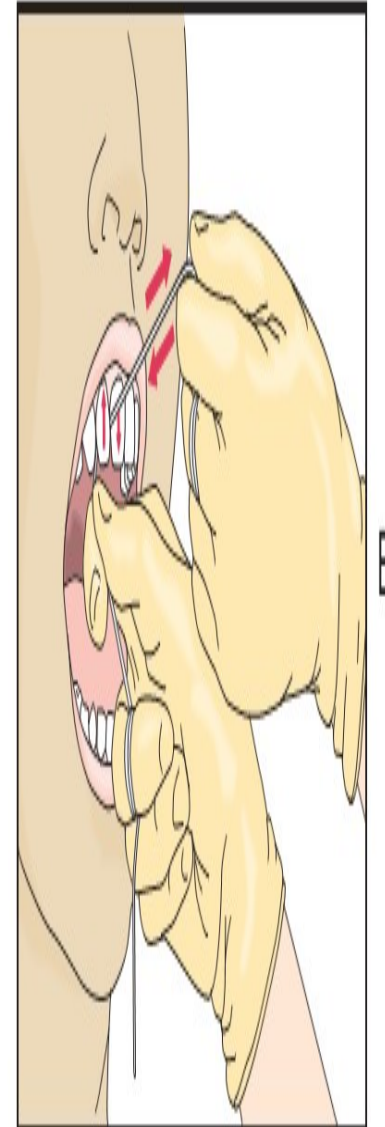
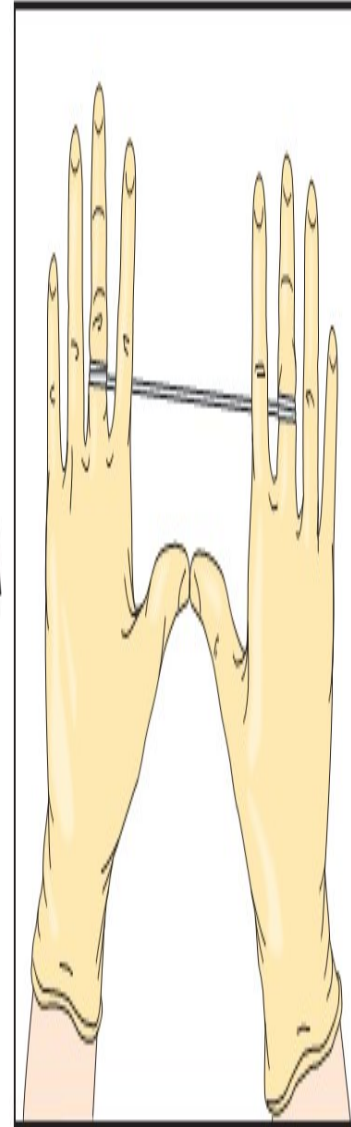
Flossing

It removes residues particles between the teeth

Technique

- ▣ Wrap one end of the floss around the 3rd finger of each hand
- ▣ To floss the upper teeth. Use the thumb and index finger to stretch the floss. Move the floss up and down between the teeth from the tops of the crowns to the gum
- ▣ To floss the lower teeth, use your index fingers to stretch the floss

▣ **Note:** If the patient has denture, remove them before starting and wash them with brush



Cont'd...

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- ☐ Give pt water to rinse the mouth and let him/her to spit the water into the basin.
- ☐ Re-comfort the pt
- ☐ Remove the basin
- ☐ Remove the towel
- ☐ Assist the patient in wiping the mouth
- ☐ Reposition the patient and adjust the bed to leave patient comfortably
- ☐ Give proper care to the equipments
- ☐ Document assessment of teeth, tongue, gums and oral mucosa.
- ☐ Report any abnormal findings

Mouth care for unconscious patient

Equipment

- ☐ Cleaning agent (check the care plan)
- ☐ Sponge swabs
- ☐ Padded tongue blade
- ☐ Water cup with cool water
- ☐ Hand towel
- ☐ Kidney basin
- ☐ Lip lubricant
- ☐ Paper towels
- ☐ Gloves

Procedure

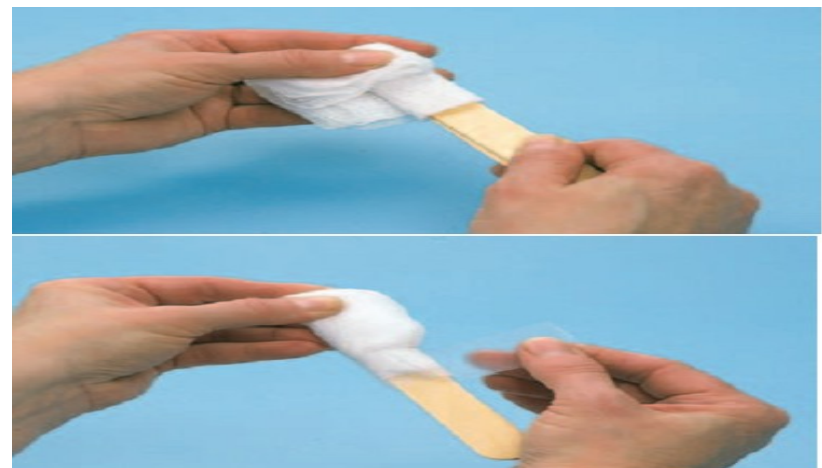
- Explain the purpose and procedure to the client or to the family
- Check client's identification and condition. Perform hand hygiene and Place the towels on the over bed table. Arrange items on top of them.
- Provide for privacy. Lower the bed rail near you if up.
- Practice hand hygiene. Put on the gloves.

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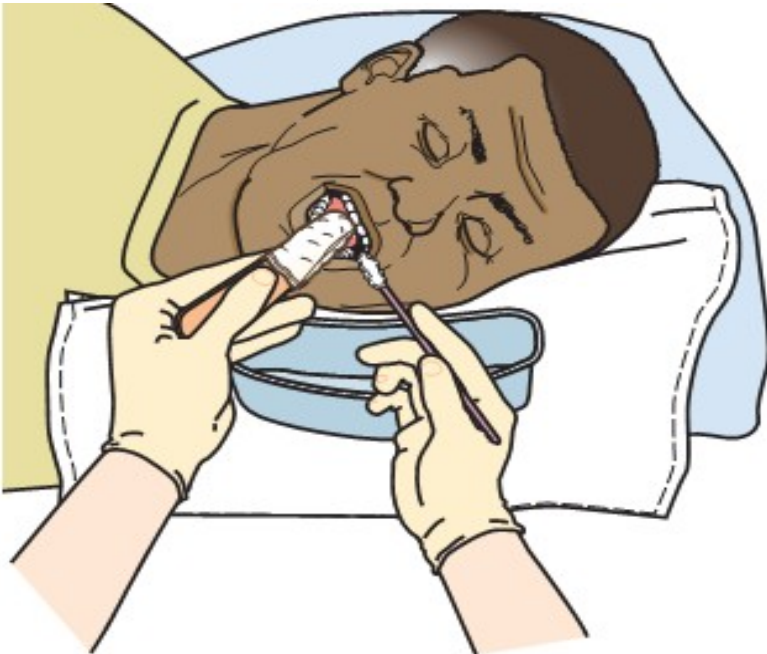
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Position the person in a side-lying position near you. Turn his or her head well to the side.

- Place the towel under the person's face.
- Place the kidney basin under the chin.
- Separate the upper and lower teeth. Use the padded tongue blade. Be gentle. Never use force.



- Clean the mouth using sponge swabs moistened with the cleaning agent (see Fig.).



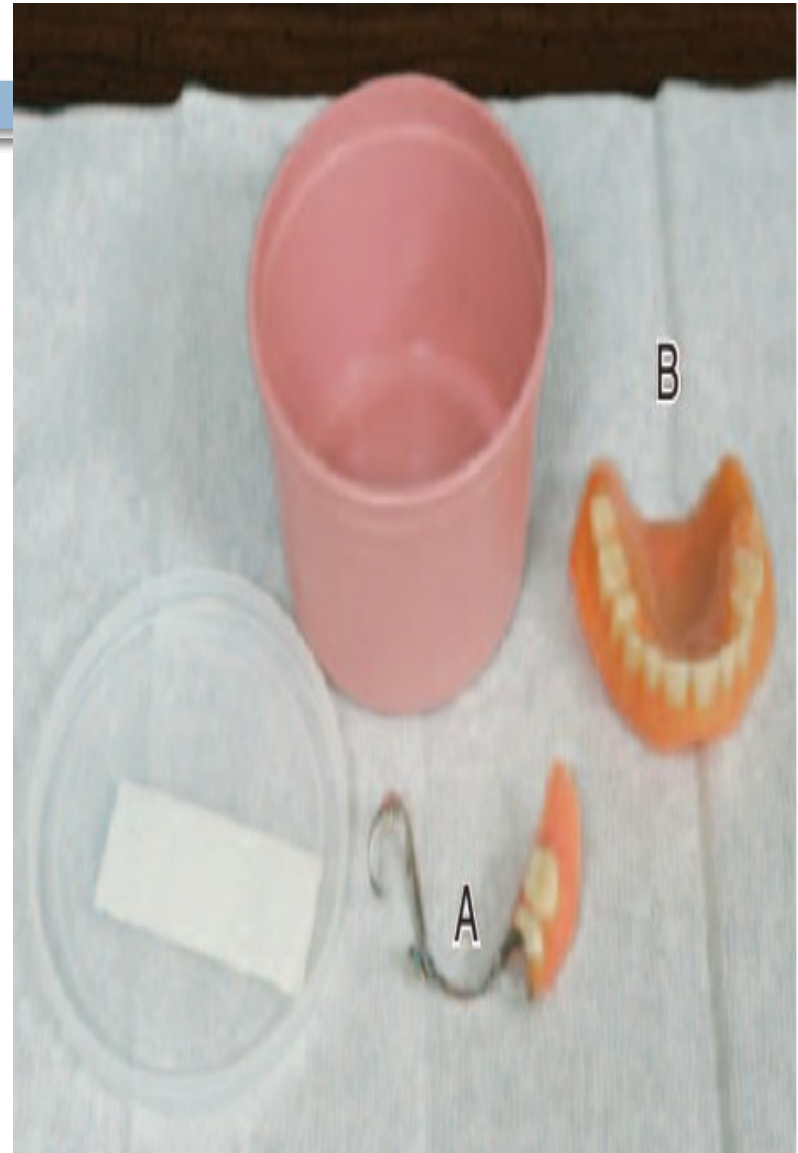
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- ☐ Clean the chewing and inner surfaces of the teeth.
- ☐ Clean the gums and outer surfaces of the teeth.
- ☐ Swab the roof of the mouth, inside of the cheeks, and the lips
- ☐ Swab the tongue.
- ☐ Moisten a clean swab with water. Swab the mouth to rinse.
- ☐ Place used swabs in the kidney basin
- ☐ Remove the kidney basin and supplies.
- ☐ Wipe the person's mouth. Remove the towel.
- ☐ Apply lubricant to the lips.
- ☐ Remove and discard the gloves. Practice hand hygiene.
- ☐ Report and record your observations.

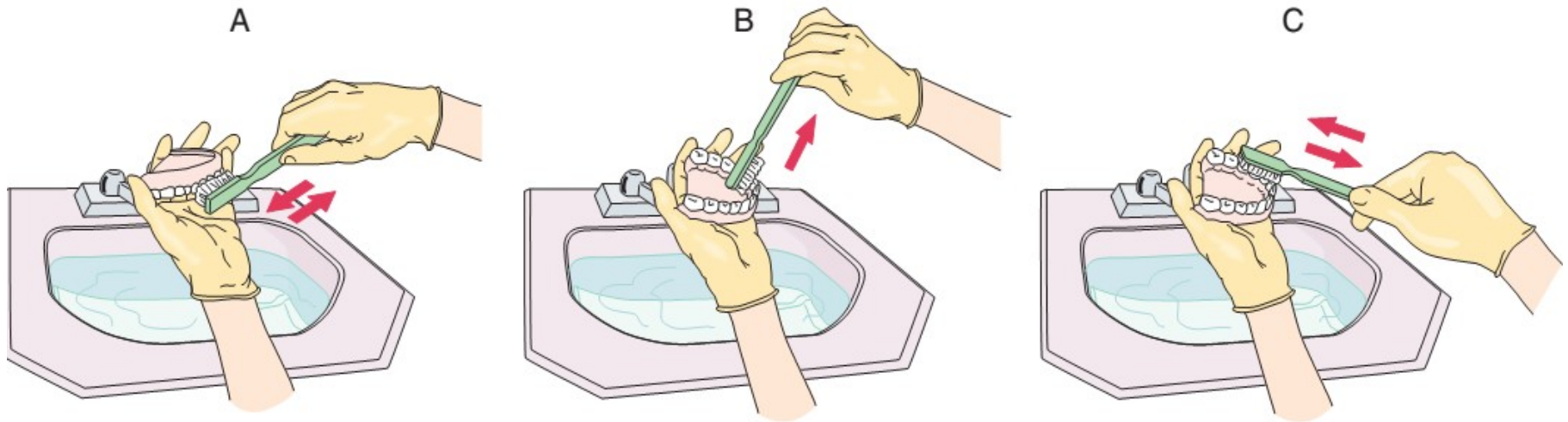
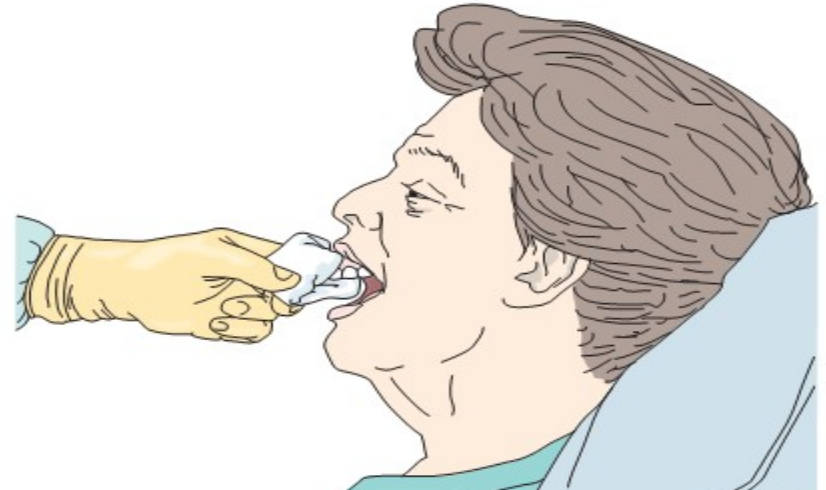
Care of denture

- A denture is an artificial tooth or a set of artificial teeth. They are often called “false teeth.” Dentures replace missing teeth. People lose teeth because of gum disease, tooth decay, or injury. Full and partial dentures are common:
 - **Full denture.** The person has no upper or no lower natural teeth. Dentures replace the upper or lower teeth.
 - **Partial denture.** The person has some natural teeth. The partial denture replaces the missing teeth



Care of denture...

- Assist the patient to remove the denture. See figure
- Place denture in denture cup or clean emesis basin take to sink and wash thoroughly using the patients brush or application. Rinse thoroughly



- Place in clean basin or denture cup and return to bedside.
- Assist the patient to rinse with mouth wash solution before dentures are replaced.
- Use tissue paper to handle dentures and assist as necessary

Offering and receiving bed pan and Urinals

- Bedpan is a material used to receive urine and feces in females and feces in male
- Urinal -is used to receive urine
- **Types of Bedpan**
- The high back, or regular pan (standard pan)
- A fracture, the slipper or low back pan



Advantage of low back pan

- Has a thinner rim than as standard bed pan
- Is designed to be easily placed under a person's buttocks

Disadvantage

- Easier to spill the contents of the fracture pan

Are useful for people who are

- ❑ Paralyzed or who cannot be turned safely (e.g. Spinal injury)
- ❑ Confined in a body or long leg cast
- ❑ Immobilized by some types of fracture
- ❑ Very thin or emaciated

The pediatric bedpan

- ❑ Are small sized
- ❑ Usually made of a plastic

Procedure

- ❑ Close curtain or door.
- ❑ Wash hands, apply gloves.
- ❑ Lower head of bed so client is in supine position.
- ❑ Assist client to side-lying position using side rail.
- ❑ Warm bedpan under warm water if needed, powder if necessary.

Cont'd...

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- Place bedpan under buttocks, with lower end near the lower back region (see Fig. A).
- While holding the bedpan with one hand, help the client to roll onto her back (see Fig. B)



- Check placement of bedpan by looking between client's legs.
- If indicated, elevate head of bed to 45° angle.
- Place call light within reach of patient; place side rails in upright position and allow for privacy.

Removing a Bedpan

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- ❑ Wash hands, don gloves.
- ❑ Lower head of bed to supine position.
- ❑ While holding bedpan with one hand, roll client to side.
- ❑ Assist with cleaning or wiping; always wipe with a front to back motion.
- ❑ Empty bedpan, clean it, and store it in proper place; if bedpan is to be emptied outside client's room, cover it during transport.
- ❑ Remove gloves and wash hands.
- ❑ Allow client to wash hands.
- ❑ Place calls light within reach; recheck that side rails are in the upright position.

Notes

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- If the individual is weak or helpless, two peoples are needed to place and remove bed pans
- If a person needs the bed pan for a longer time periodically remove and replace the pan to ease pressure and prevent tissue damage
- Metal bed pans should be warmed before use by
- Running warm water inside the rim of the pan or over the pan
- Covering with cloth

- Semi-Fowler's position relieves strain on the client's back and permits a more normal position for elimination. Improper placement of the bedpan can cause skin abrasion to the sacral area and spillage
- Place a regular bed pan under the buttocks with the narrow end towards the foot of the bed and the buttocks resting on the smooth, rounded rim
- Place a slipper (fracture) pan with the flat, low end under the client's buttocks
- Covering the bed pan after use reduces offensive odors and the clients embarrassment

Giving Perineal Care

Definition

- Perineal care (pericare) involves bathing or cleaning the genital and anal or surrounding areas. These areas provide a warm, moist, and dark place for microbes to grow
- Cleaning prevents infection and odors, and it promotes comfort
- Perineal care is done daily during the bath
- It also is done whenever the area is soiled with urine or feces.

Indication

=Perineal care is very important for pt who:

- ☐ Have urinary catheters
- ☐ Have had rectal or genital surgery.
- ☐ Are menstruating
- ☐ Are incontinent of urine or feces.
- ☐ Are uncircumcised.
- Being circumcised means that the fold of skin (foreskin) covering the glans of the penis was surgically removed.
- Being uncircumcised means that the person has foreskin covering the head of the penis

- The person does perineal care if able. Otherwise, it is given by the nursing staff
- This procedure embarrasses many people and staff, especially when it involves the other sex.

Purpose:

- To keep cleanliness and prevent from infection in perineal area
- To make him/her comfortable

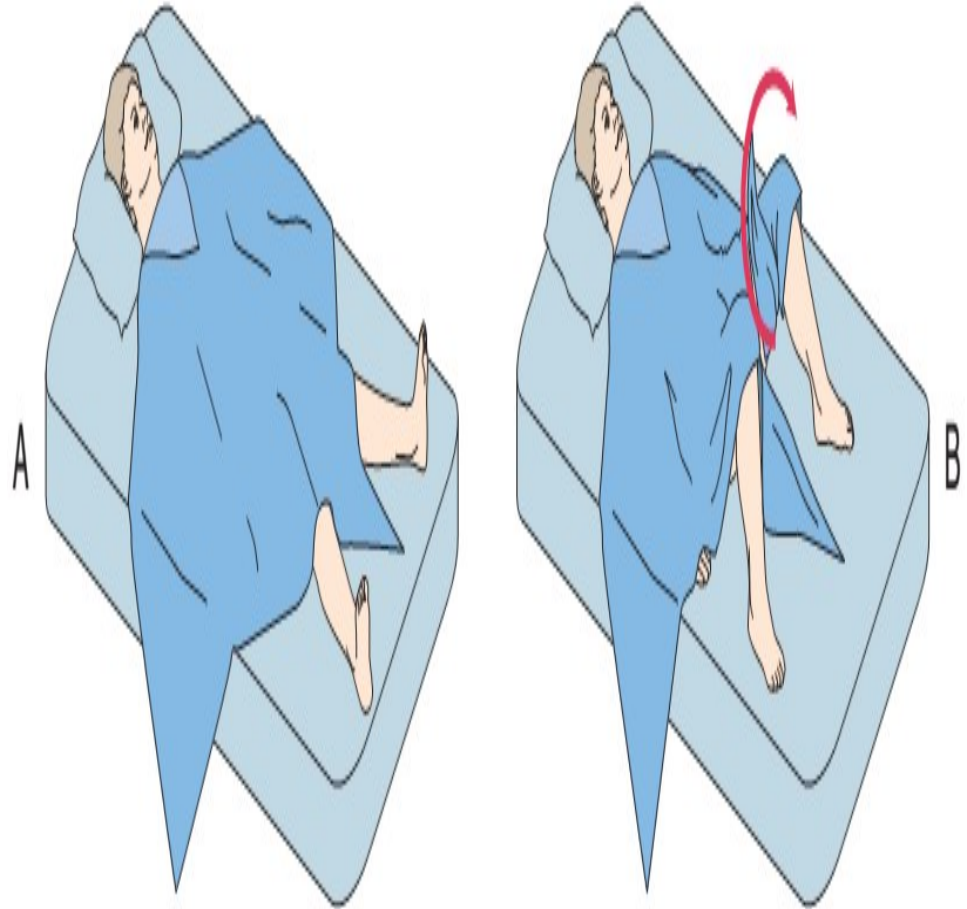
Equipments required

- ▣ Gloves(non- sterile) (1 pair)
- ▣ Sponge cloth (1)
- ▣ Basin with warm water (1)
- ▣ Waterproof pad or gauze
- ▣ Towels (1)
- ▣ Mackintosh (1)
- ▣ Soap with soap dish (1)
- ▣ Toilet paper
- ▣ Bed pan (1): as required

Procedure

- ▣ Provide adequate explanation about the procedure to the client.
- ▣ Hand washing .Gather all required equipments. Bring to the bed side
- ▣ Close the door to the room and place the screen to provide privacy
- ▣ Perform hand hygiene and put on gloves
- ▣ Raise the bed to a comfortable height if possible.

- Preparation the position:-
- Uncover the client's perineal area.
- Drape the person as in Figure



Cont'd...

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- Fill the wash basin. Water temperature is usually 105°F to 109°F (40.5°C to 42.7°C)
- Follow the care plan for water temperature. Measure water temperature according to agency policy
- Ask the person to check the water temperature. Adjust the water temperature if it is too hot or too cold.
- Help the person flex her knees and spread her legs Or help her spread her legs as much as possible with the knees straight.

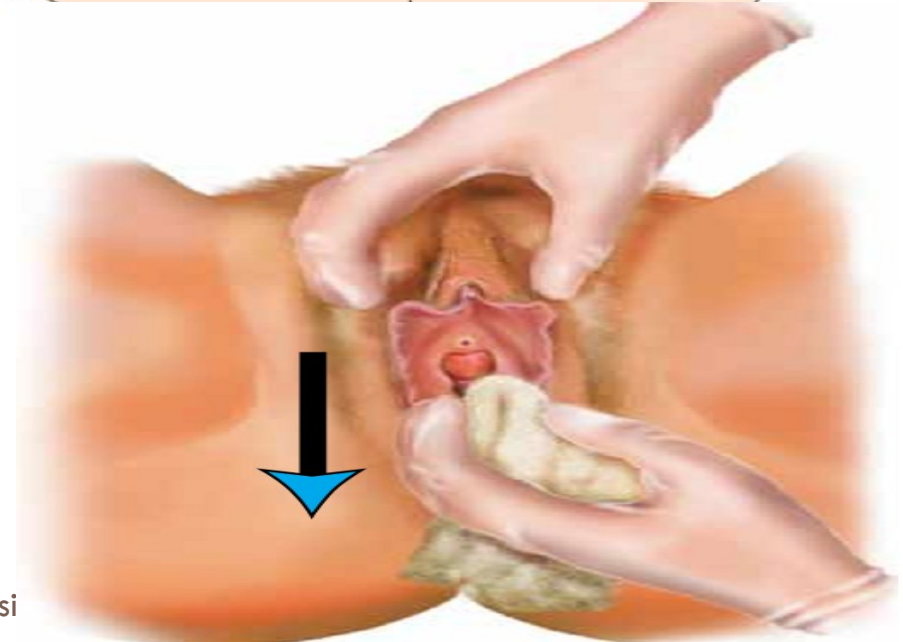
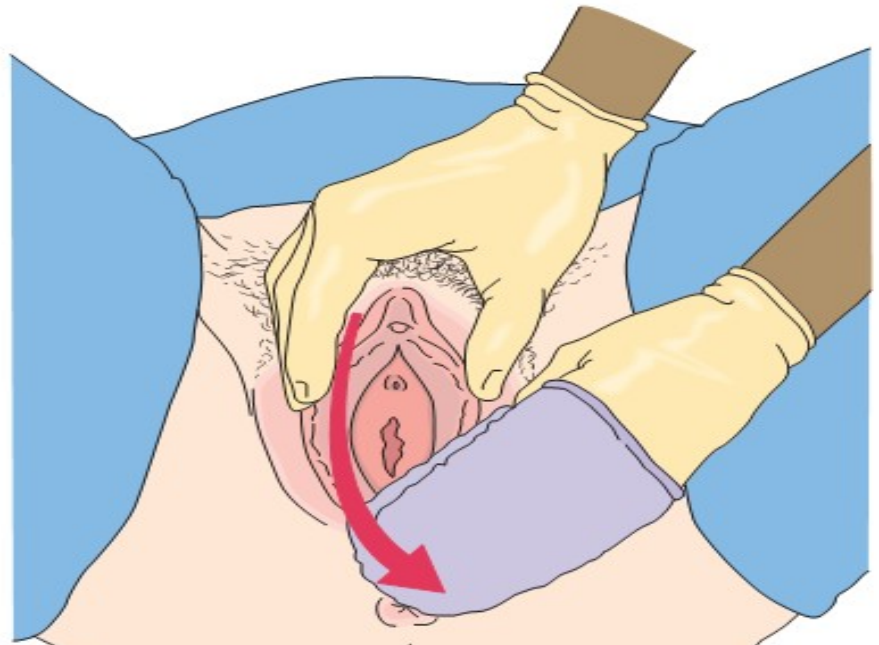
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- ☐ Place a waterproof pad under her/his buttocks
- ☐ Remove any wet or soiled incontinence products
- ☐ Fold the corner of the bath blanket between her legs onto her abdomen.
- ☐ Wet the washcloths. Squeeze out excess water from a washcloth
- ☐ Make a mitted washcloth. Apply soap.
- ☐ Cleanse the thighs and groin:
- ☐ Make a mitt with the sponge cloth.
- ☐ Cleanse the client's upper thighs and groin area with soap and water.
- ☐ Rinse and dry.
- ☐ Wash the genital area next.

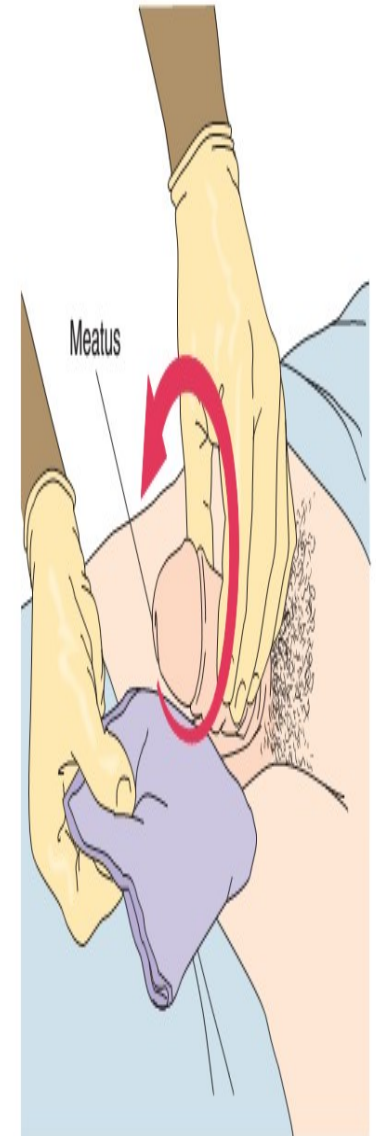
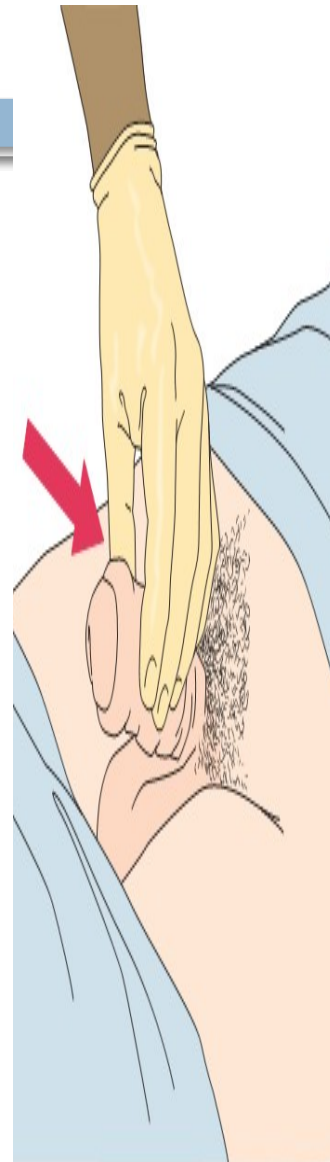
Female client:

- Use a separate portion of the sponge towel for each stroke
- Change sponge towel as necessary.
- Separate the labia and cleanse downward from the pubic to anal area.
- Wash between the labia including the urethral meatus and vaginal area.
- Rinse well and pat dry.

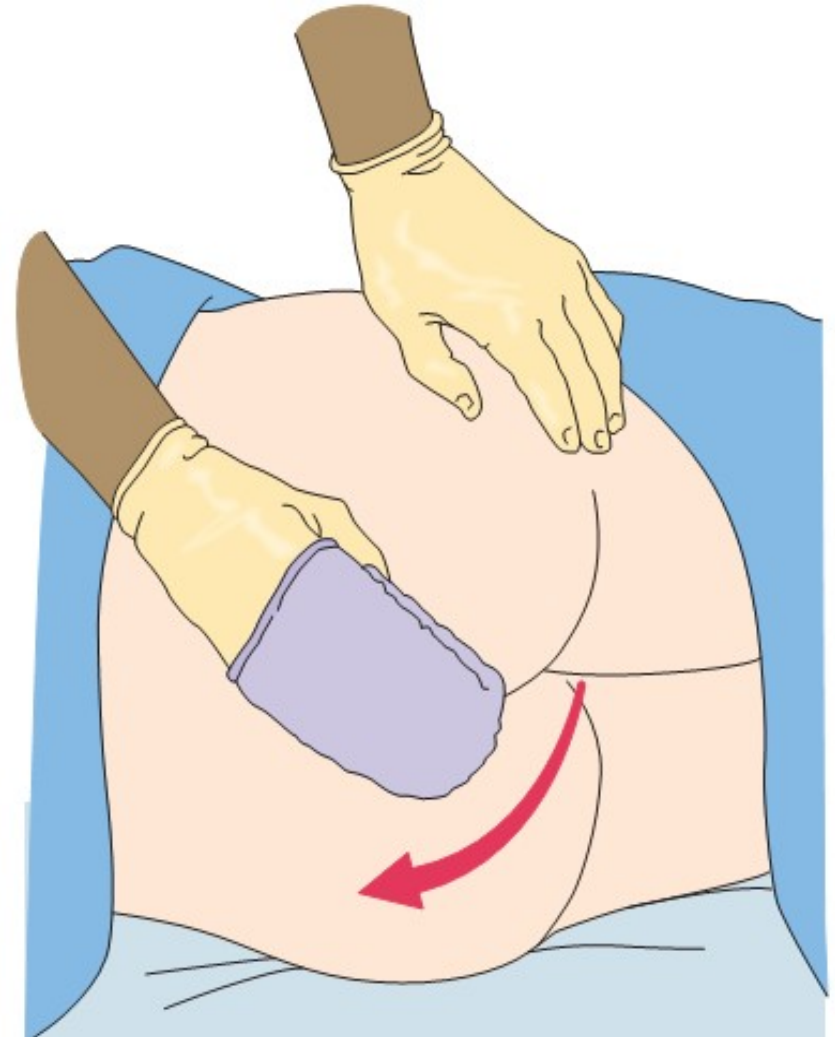


Male Client:

- Gently grasp the client's penis.
- Cleanse in a circular motion moving from the tip of the penis backwards toward the pubic area
- In an uncircumcised male, carefully retract the foreskin prior to washing the penis.
- Return the foreskin to its former position.
- Wash, rinse, and dry the scrotum carefully.



- Assist the client to turn on the side. Separate the client's buttocks and use toilet paper, if necessary, to remove fecal materials.



- Cleanse the anal area, rinse thoroughly, and dry with a towel. Change sponge towel as necessary.
- Apply skin care products to the area according to need or doctor's order.
- Return the client to a comfortable position.
- Remove gloves and perform hand hygiene.
- Document the procedure, describing the client's skin condition. Sign the chart

Feeding a helpless patient

- During illness, trauma or wound healing, the body needs more nutrients than usual
- However, many peoples, b/c of weakness, immobility and/or one or both upper extremities are unable to feed themselves all or parts of the meal.
- Therefore, the nurse must be knowledgeable, sensitive and skillful in carrying out feeding procedures.

PURPOSE

- To be sure the pt receives adequate nutrition
- To promote the pt well-beings

PROCEDURE

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1. Prepare pt units

- Remove all unsightly equipments; remove solid linens and arranging bedside tables.
- Control unpleasant odors in the room by refreshing the room. Oder free environment makes eating more pleasant and aids digestion.

Cont'd...

125 2. Prepare the patients

- ❑ Offers bedpan and urinals. To comfort pt and avoid interruption by elimination needs.
- ❑ Assist pt to wash hands, face and oral care
- ❑ Position patient comfortably ⇒ Mid or high Fowler's position
- ❑ Protect the bed using suitable protective cover



3. Prepare the food tray

- Identify the types of diet ordered.
- Assess any special conditions in which the pt delayed or omitted (e.g. Lab, radiologic examination or surgery)
- Assess any cultural or religious limitations, specific likes or dislikes.
- Obtain any special utensils that you planned to use

4. Feed the patient

- Place the food tray in such a way that the patient can see the food.
- Position yourself at pt's eye level, if at all possible
- Never hurry a pt's eating. This can make pt uncomfortable and fearful of taking up your time.
- Allow pt to determine when enough has been eaten, as way of providing choices.

5. Comfort patient

- ☐ Assist hand washing and oral care
- ☐ Offer bedpan and commodes, of indicated
- ☐ Comfort patient, provide quite environment so that the pt may relax after meal, which also promote good digestion.

6. Care of equipment and Document feeding and any assessment.

THANK YOU SO MUCH!!

Let's Read to Make a D/ce



**" Integrity is better capital than money.
You can accumulate it just like money,
and you can use it just like money,
but it goes further, and is enduring. "**

Steve Wozniak